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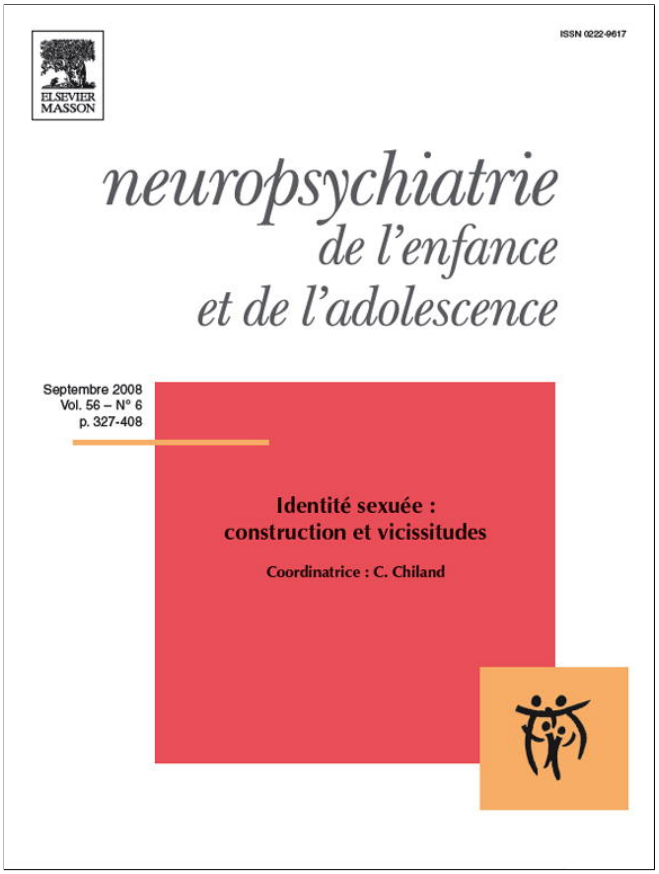

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Original article

Working at the edge: Engaging in therapeutic work with young people with atypical gender identity development

Working at the edge : s'engager dans un travail thérapeutique avec des jeunes ayant un développement atypique de l'identité de genre

D. Di Ceglie *

Gender Identity and Adolescent Department, Tavistock clinic, London, United Kingdom

Abstract

Professionals working in a service for young people with Gender Identity Disorder (GID) often experience a particular type of pressure in the course of the work which makes thinking and decision-making very difficult. The paper suggests that an examination of the position of the professional and of the service, within the system may bring some relief and be a starting point to direct thinking and possibly action. The perceived position of the professional and of the Gender Identity Development Service within the wider organization of a psychotherapy and mental health trust is discussed. The paper makes reference to the model developed by Rom Harre et al. as described by Campbell and Groenbaek in their book *Taking positions in the organisation*. The position taken by the professional or the service is also explored as a possible re-enactment of the psychological dynamics of the young people with GID and their families. This view is based on Britton's paper on the re-enactment of individual and family dynamics in the professional network. A view is put forward that this dynamic leads the professional or the service to the risky position of "working at the edge". The paper discusses processes which could facilitate the move from a position of being psychologically at an edge to the more secure position of being in the mainland. This involves exploration of the model of care and the relationship of the professional to the wider organization in which the service is located and to self-help organizations.
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Keywords: Gender identity disorder; Children; Adolescents; Impact on professional and service

Professionals working in a service which works with children and adolescents presenting with the clinical features of a Gender Identity Disorder (GID), often experience a particular type of pressure during the course of their work. This makes thinking and decision-making very difficult. They feel at risk of becoming involved in courses of action which could be unhelpful to the children and particularly adolescents they are trying to help. Occasionally, the pressure is generated by other parts of the system in which they are involved such as user groups, management or the media. In these circumstances a professional may get some relief by defining which position he/she has ended up taking within the system. Being unaware of one's position may perpetuate the sense of pressure and anxiety, while the acknowledgement of one's position can become a good starting point to direct thinking and possibly action.

In this respect, Positioning theory as described by Langenhove and Harre [1] provides a useful model.

Campbell and Groenbaek draw from this theory in their book *Taking positions in the organisation* ([2], p.16). They say:

> "Generally, we choose to take positions within discourses, and we are also positioned by others on the basis of what we say and do. . . These different positions can be placed on one continuum, or what we call a semantic polarity — *semantic* because it provides the basis for meaning."

One example of this polarity is feeling insecure and feeling secure and the individual can position himself/herself within this spectrum.

Feeling insecure — Feeling secure

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* Gender Identity Development Unit, Tavistock and Portman NHS Foundation Trust, 120, Belsize lane, London NW3 5BA, United Kingdom.

E-mail address: gidu@tavi-port.nhs.uk.

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Fig. 1. Niagara Falls.

Fig. 2. Individual walking on the edge of a cliff top.

In the work with children and adolescents I would like to suggest the use of the geographical polarity: Edge and Mainland and apply this in a metaphorical way.

Edge Mainland

Images which evoke the experience of being at an edge could be the Niagara Falls (Fig. 1) or of an individual walking on the edge of a cliff top (Fig. 2). Another image could be *The Creation of Adam* by Michelangelo (Fig. 3); an edge between the two figures of Adam and God. Another representation of the edge can be seen in the powerful "Chicken Run" scene of the film *Rebel Without a Cause*. It portrays the race between two young people towards a cliff edge and the catastrophic fall of one of them over the edge.

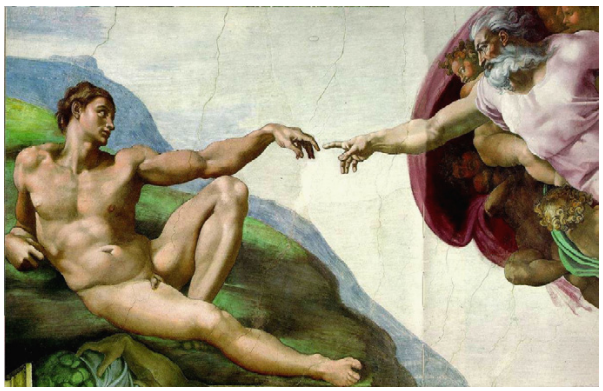

Fig. 3. *The Creation of Adam* by Michelangelo.

All these images can conjure up a constellation of feelings experienced by people who perceive themselves to be emotionally at the edge. It is often an experience of being at the boundary between life and death and of being at a high level of risk in relation to ones own physical and psychic survival.

1. The metaphor of the edge

The metaphor of the edge can be a representation of:

- the boundary between the body and the mind;
- the pressure to readjust the body to a gender identity which is in contrast with the physical appearance of the body;
- the sense of living at the edge and of impending catastrophe if the realignment of the mind and body does not occur quickly enough;
- the edge between life and death – the high risk of self harming that some young people with GID present;
- living at the edge of society experiencing stigma and often bullying.

Campbell and Groenbaek ([2], p. 18) describe a process in which the professional is being positioned by others. They consider it complicated, but also crucial to the understanding of group interactions. In our case the professional often ends up experiencing a similar positioning to that of the adolescent, and working in this area often feels like being at an edge.

The service is also at times positioned at the boundary of the organization within which it is located and at risk of being pushed out and disappearing under the pressure generated by the work with these young people and their families.

Campbell and Groenbaek do not take into consideration in their book the contribution that unconscious processes and communication can make to the mirroring of the young persons states of mind in the mind of the professional.

R. Britton in his paper *Re-enactment as an unwitting professional response to family dynamics* ([3], p. 48) describes this process in detail.

He suggests:

> "that contact with some families may result in professional workers or their institutions becoming involved unknowingly in a drama which reflects a situation in the relationships of the family or within the minds of some of its individual members; and this is not recognized but expressed in action".

Splitting and projective identification are the mechanisms involved in the communication of these states of mind. It is beyond the scope of this paper to describe in detail these complex processes. It is important to highlight here how being positioned at the edge is the result of the reflection in the professional of states of mind, often present in the young person and their family. The awareness of these processes is important to help young

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people to deal with these states of mind and therefore to be able to move from a position of being psychologically at the edge to the more secure position of being in the mainland. In order to facilitate this process three areas need to be considered by the professional and the service involved. They are:

- the model of care;
- the relationship of the professional to the self-help organizations;
- the relationship of the professional to the wider organization in which the service is located.

2. The model of care

The causation of GID is unclear and probably multifactorial. At the core of gender dysphoria, there is an issue of identity. Young people are very sensitive and feel intruded upon by anyone attempting to change who they feel they are. At the GID Service, where I work, we have developed a model of management in which altering the GID *per se* is not a primary therapeutic objective. The aim of the service is to assist the development of these young people so that they do not acquire other problems. The aim is to enable families to understand that these children feel differently from the majority of the other children in their school. Our other aim is to allow the children to communicate their feelings about gender identity openly and to break the cycle of secrecy that surrounds these types of issues. Another aim is to enable the exploration of different possibilities during the course of development and reduce the isolation the child experiences at school. It is also important to enable the child and the family to tolerate some uncertainty about the outcome of the gender identity development. These aims are listed in Box 1 and I have previously described them in detail [4].

Box 1: Primary therapeutic aims [4]
To foster recognition and non-judgmental acceptance of gender identity problems
To ameliorate associated behavioral, emotional and relationship difficulties [5]
To break the cycle of secrecy
To activate interest and curiosity by exploring the impediments to them
To encourage exploration of the mind-body relationship by promoting close collaboration among professionals in different specialities, including a pediatric endocrinologist
To allow mourning processes to occur [6]
To enable symbol formation and symbolic thinking [7]
To promote separation and differentiation
To enable the child or adolescent and the family to tolerate uncertainty in gender identity development
To sustain hope

After an assessment these aims could be achieved through:

- family therapy;
- child psychotherapy and parental support/counseling;
- consultation to the network with or without further direct involvement with the young person and their family;
- intermittent reviews to monitor gender identity development;
- group work for parents;
- occasionally, referral to the Pediatric Liaison Clinic for physical assessment;
- a combination of the above.

Another task of the therapeutic input is to evaluate the persistence or the desistence of the atypical gender identity. In other words, it is important to evaluate in a collaborative effort with the adolescents whether or not the beliefs about oneself are well established and unlikely to change.

Alternatively, the adolescent perceptions may not be strongly held beliefs but the results of imagination and therefore more fluid and variable. In his book *Beliefs and imagination* Britton ([8], p. 11) says:

> "Beliefs have consequences: they arouse feelings, influence perceptions and promote actions. Phantasies, conscious or unconscious, which are *not* the object of belief, do not have consequences: disavowal therefore can be used to evade these consequences".

Adolescents who have a persistent experience of incongruity between mind and body find puberty painful and unbearable. They are often at high risk of suicide attempts. A staged approach to management has been devised and this provides a containing framework for these unbearable states of mind [9]. This approach involves four stages:

- stage one involves a therapeutic exploration;
- for adolescents whose GID persists and becomes more distressing during adolescent development stage two may be considered following careful assessment. This involves the use of hypothalamic blockers which suppresses this production of oestrogen or testosterone and produces a state of biological neutrality. This intervention is reversible. A controversy exists about the timing of this intervention during pubertal development;
- stage three includes partially reversible interventions such as hormonal treatment which masculanizes or feminizes the body. Current guidelines allow this intervention after the age of 16;
- finally, in adulthood after the age of 18, stage four could be considered. This includes irreversible interventions, such as surgical procedures.

This model provides a safe journey towards harmonizing mind and body as it provides a space for reflection and thinking at each stage.

> "A follow-up study of transsexual adolescents who, after careful assessment, started the process of sex reassignment

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during adolescence (after the age of 16) shows that they had achieved a good level of psychological and social adjustment at least 1 year after surgical intervention" [10].

3. The relationship of the professional to the self-help organizations

Self-help organizations can be another source of support to adolescents and families in distress. They can provide a complementary form of help to that of the professional as they can become much more involved in practical help and reduce the sense of isolation that these families often experience. A self-help organization, called Mermaids, was formed by parents who attended a group run at the GID Service. The suggestion of running a group in my service came from a parent of a child who presented with gender identity problems. The child had been referred for an expert opinion in court proceedings, where the parents were in dispute over custody and were blaming each other for having caused the child's problem. During an attendance at court the mother talked about her feelings of isolation. She asked if she could be put in touch with another parent facing similar problems. It was from this request that the idea of running a group evolved. We developed a model for running this group for parents which has been described in a paper [11].

As our groups are time limited, some parents decided to form an independent organization that could offer ongoing help in a more informal setting. Representatives from the staff of the gender identity unit attend committee meetings and the annual general meeting. Our professional input is to enable the group to accept a variety of views coming from different parental and young people's perspectives. There is often the risk in groups of becoming dominated by a particular point of view at the exclusion of others. This impedes constructive debates in the search for solutions which take into account differing individual needs. The role of the professional is to endeavor to reduce the potential for splitting and the risk of destructive confrontation which can develop within a group, as this is unhelpful to the task of supporting children and adolescents who are in distress.

4. The relationship of the professional to the wider organization in which the service is located

In order to operate effectively the professional in the service needs to be supported by the organization in which they belong. In our service it became important that our protocol was authorized and supported by the appropriate professional committee. This gives the professional working within the service a sense of security which can in turn be communicated to the users of the service. The need for authorization and support which professionals working in these areas have can lead to conflict with self-help organizations. They may feel that particular practices should be changed more quickly than the complex scrutiny of a wider organization allows. So the professional may feel pushed from a position of security in the mainland towards the risky edge of the earlier metaphor. A capacity for negotiation in the professional is required if he or she has to move to a more secure position which would in turn offer security and stability to the young person and their family.

5. Conclusions

In this paper I have tried to describe the frequent experience that professionals working in the area of atypical gender identity have of being at the edge. This is experienced as risky and anxiety provoking. I have tried to show how a move to a more secure base may include the clear definition of a path to follow and agreed therapeutic aims with the user of the service. A continuous process of negotiation is also required with user organizations and the wider institutions where the professional and service belong.

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